

إِسْم الْمَرِيضِ رُبَاعِيًّا : إِيْمَانُ عُوَيْسِ عِيْدِ عَلِي

الحى / المدينة : العياط

المحافضة : الحيز ه

رقم البطاقة : 28012012100845 رقم التلفون : 01112250948

العمر(تاريخ الميلاد) : 01-12-1980 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضى الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157]

التعليق على التشخيص : -----

الاسم: د/محمد كيلاني
 الاسم: د/ندى حسن
 الاسم: د/نرمين رمضان
 الوظيفة: _____
 الوظيفة: _____
 الوظيفة: _____
 التوقيع: _____
 التوقيع: _____
 التوقيع: _____
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
 تعتمد صحة البيانات المرفقة
 رئيس اللجنة ومدير
 المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د.ا ----- / د.ا

قرار اللجنة

التشخيص النهائي :

خطـة العـلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: إيمان عويس عيد علي National ID: 28012012100845	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Dms
سالم



MDT FORM 3/12/2025 G

Name	إيمان عيسى علي	MRN	3097125
Age	44y	ID	0112950948
Menopausal status	Peri-menopausal	Phone	01226680927
NRS SCORE		Marital status	Married
BMI		Family history	-ve
CO morbidity	Free	REASON OF MDT	New

Clinical Data

BX:	Left IDC II of Low grade DCIS		
ER:	PR:	HER 2:	Ki67:
818	818	+1	30%.

Sonomammography: ACR B.
Lower Central, Middle third

Mets Workup: Lesion is suspicious
microcalcifications within
2.5mm deep to skin 2.8 x 1.1 cm.
one pathologic LN

TNM
210 Transvaginal
US
Requested

Recommendations: CT → Bulky uterus
Heard thickening likely inflammatory, Rt hepatic
Basal lesion
For possibility of up-front left mastectomy
US neo 2 (Small Sted)

MDT OUTCOME & PLAN

1) DCIS excision
2) NACT: ACX3. Tox 90
3) Left WLE + SLNB acc to Response

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
		Mona Taly			

Patient Name : الأستاذة / ايمان عويس عيد على	Request No. : 13006319
Patient No. : 28012012100845	Request Date : 14/10/2025
Path No. : 3142/25	Verification Date : 21/10/2025
Sex : Female	Financial Category : جوستاف روسي - مبادره
Age : 44 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected on 13/10/2025.

Gross Examination:

- Four grey white tissue cores, the longest was 1 cm. Totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
- * DCIS: 1% Low grade ductal carcinoma insitu (Solid pattern).
- * Angioinvasion: Present.
- * Perineural invasion: Absent.
- * Microcalcification: Present.
- * IPT (Positive control showed appropriate reactivity):☐
- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67%: 30%.

Diagnosis:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 2, with 1% Low grade ductal carcinoma insitu (Solid pattern).

Verified by :

Dr.Duaa Mohamed

Printing Date : 26/10/2025 10:40:05

Page 1 of 3

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / ايمان عويس عيد على	Request No. : 13006319
Patient No. : 28012012100845	Request Date : 14/10/2025
Path No. : 3142/25	Verification Date : 21/10/2025
Sex : Female	Financial Category : جوستاف روسي - مبادره
Age : 44 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Low (score 1+).
- Ki67%: 30%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed



Patient Name: Eman Owais Eid Ali	Date : 02/10/2025
Exam : MAMMOGRAPHY BOTH	Age : 44 Year(s)
Medical ID: 8590	Pat. ID: 121883

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 44-year-old female with negative family history complaining of left breast lump of few months duration.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- Lower central, middle third, irregular speculated high density mass lesion with suspicious microcalcifications within.
- **Ultrasound** demonstrated 6 o'clock, middle third, 2.5 mm deep to the skin, irregular heterogenous hypoechoic solid mass lesion with partially speculated partially micro-lobulated margins with surrounding echogenic halo, posterior acoustic shadowing and mild internal vascularity on color Doppler study as well as small cystic degeneration measuring **28 x 17 mm.**
- Normal skin/nipple/areola complex.
- One pathological axillary level 1 LN is noted showing focal cortical thickness reaching 13 mm.

Conclusion:-

Right breast (BIRADS 1): Normal study.

Left breast (BIRADS 5): 6 o'clock suspicious mass lesion with ipsilateral one pathological level 1 axillary lymph nodes.

Recommendation:-

For US guided Tu-cut biopsy and histopathological correlation.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations

Suspected Cancer Risk

BIRADS 0: Non-informative study.	0 %
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	2 %
BIRADS 3: Probably benign, for close follow up.	50 %
BIRADS 4: Probably malignant, for biopsy.	
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Patient Name: Eman Owais Eid Ali	Date : 10/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 44 Year(s)
Medical ID: 3097/2025	Pat. ID: 121883

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported case of left breast carcinoma.

(I) Chest:

- Patent central air ways with well inflated both lungs.
- Both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Left LIQ breast lesion , measuring 28x20 mm with left axillary pathological lymphadenopathy , measuring 18x12 mm ,for sonomammographic correlation.

(II) Abdomen and pelvis:

- The liver is average in size, showing homogenous CT density with right hepatic hypodense focal lesion, at segment 8 , measuring 8.6x6.2 mm in diameter with isodensity in the delayed phases.
- Mild circumferential mural thickening of the appendix and the adjacent terminal ileum with mild smudging of the surrounding fat planes.
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.

- Bulky uterus with IUCD.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.

OPINION:

Clinically reported case of left breast carcinoma showing :

- No CT evidence of pulmonary deposits.
- Right hepatic lobe benign benign-looking focal lesion ,likely focal; fatty infiltration.
- Bulky uterus, for sonographic assessment.
- Mild terminal ileal and appendiceal wall thickening, likely inflammatory, for laboratory correlation.

Dr.Sara Mahmoud, MD

Dr.Sara Ali

Dr.Sara Ali



د. يوسف
روسي
9-5

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

السلطة المصرية
لصحة المرأة

09/20/2026

MDT FORM

Name:	Eman Eweis Eid .	MRN:	2603
Age:	46 Y	ID:	28012012100845
Menopausal Sta	pre	Phone:*	01112250948
NRS SCORE:		Marital Status:	
BMI:		Family History:	-ve fh
Gender		Reason Of MDT:	
CO Morbidity:	m,free	post op	
Surgical History:*	nad	ECOG Performance Status	0 0 1 0 2 0 3 0 4

Clinical Data:

BX: **ER:** 8/8 **PR:** 8/8 **HER2:** +1 **KI67:** 30

History:

SP It bcs + slnb post nact
ldc g2
dcis low grade
skin free
deep margin positive for low grade ductal carcinoma in situ +
NEARBY CAUTERIZED 0.3 CM FREE OF TUMOR
free other surgical margins
slnb 1/9
stage ypt2,ypn1a

Sonomammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

Dynamic breast MRI + C (to assess any role for re-resection)
If not for re-resection --> PORT
Femara + zoladex + zometa / 6 months

T N M

R L



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. خالد عبد العزيز	د. محمد علي لوي		